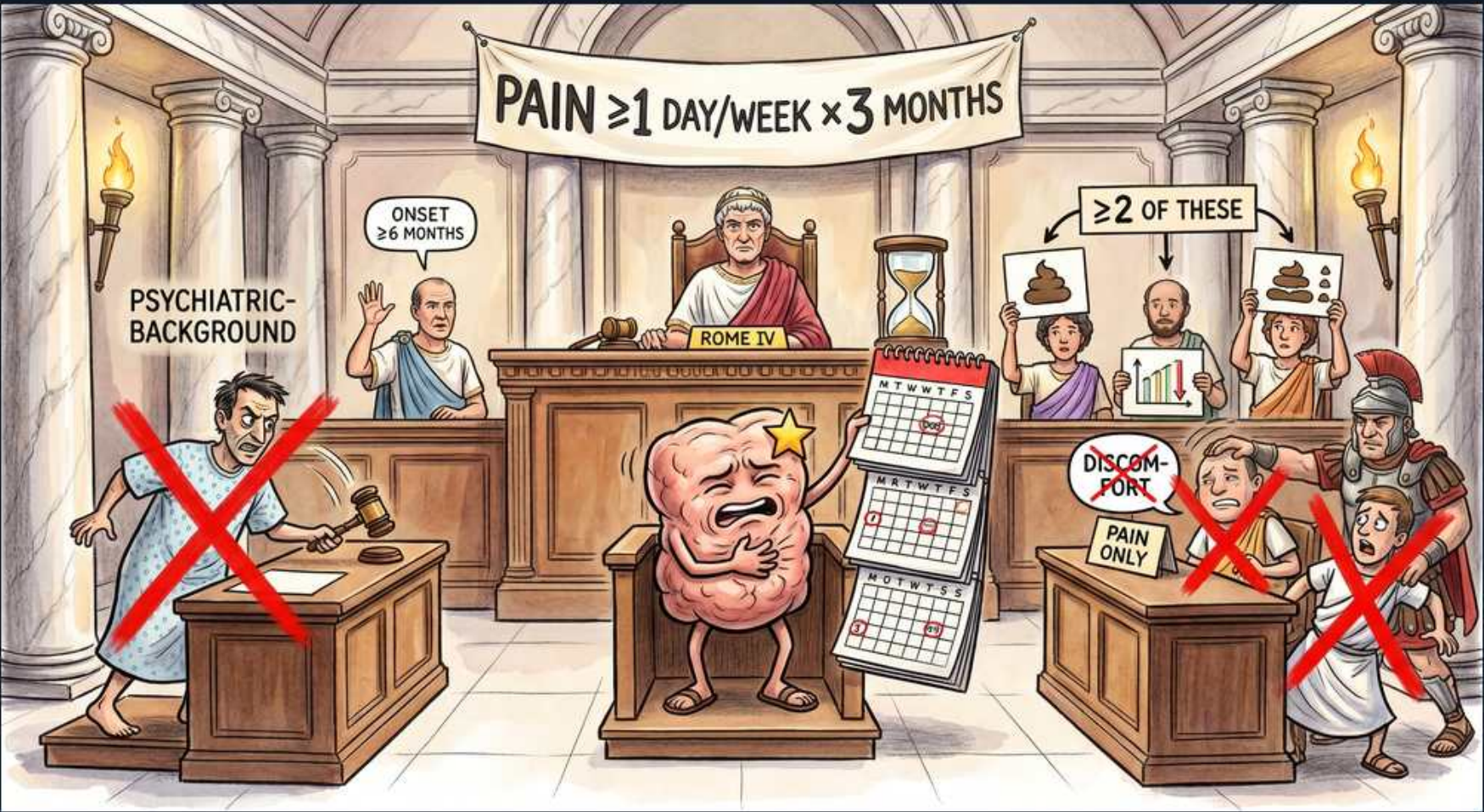




1. PAIN ≥1 day/week × 3 months banner

WHAT YOU SEE

Top banner reading 'PAIN ≥1 DAY/WEEK ×3 MONTHS'.

WHAT IT ENCODES

Rome IV essential criterion: recurrent abdominal PAIN at least 1 day per week, on average, for the last 3 months. This is the mandatory entry gate — without pain there is no IBS diagnosis. Pain frequency (≥1 d/wk) and duration (≥3 mo) are both required.

2. Onset ≥6 months

WHAT YOU SEE

Standing Roman holding a sign 'ONSET ≥6 MONTHS'.

WHAT IT ENCODES

Symptom onset must date back at least 6 months before diagnosis, even though the active-symptom window assessed is the most recent 3 months. Distinguishes chronic functional disease from acute self-limited GI complaints.

3. Rome IV bench (judge)

WHAT YOU SEE

Judge presiding behind a bench labeled 'ROME IV'.

WHAT IT ENCODES

Rome IV (2016) is the governing diagnostic framework. Key change from Rome III: 'discomfort' was REMOVED and frequency tightened to ≥1 day/week. IBS is a clinical (positive-criteria) diagnosis, not a diagnosis of exclusion.

4. Crying gut defendant

WHAT YOU SEE

An anthropomorphic intestine with a gold star, crying and clutching itself.

WHAT IT ENCODES

The gut character IS the IBS patient — the painful gut is the protagonist. Visceral hypersensitivity and altered gut-brain motility drive the pain. The gold star marks it as the legitimate/correct central element.

5. Pain calendar

WHAT YOU SEE

A wall calendar with red X marks scattered across days.

WHAT IT ENCODES

Pain recurs on ≥1 day/week averaged over 3 months — visualized as recurrent marked days. Frequency is averaged, so it need not be every week but must reach the weekly threshold on average.

6. ≥2 of these sign

WHAT YOU SEE

Sign reading '≥2 OF THESE' over the right-side jurors.

WHAT IT ENCODES

Beyond pain, you need ≥2 of 3 supportive features: (1) pain related to defecation, (2) associated change in stool FREQUENCY, (3) associated change in stool FORM/appearance. These define the syndrome alongside the mandatory pain.

7. Defecation/stool jurors (poop signs)

WHAT YOU SEE

Three jurors holding up signs with stool symbols and a frequency graph.

WHAT IT ENCODES

The three Rome IV add-on criteria: relation to defecation (pain better or worse with passing stool), change in stool frequency, change in stool form (Bristol scale). Need any 2 of these 3 plus the pain criterion.

8. Psychiatric background (red X)

WHAT YOU SEE

A figure on the left dragged out, crossed with a large red X, labeled 'PSYCHIATRIC BACKGROUND'.

WHAT IT ENCODES

Although anxiety/depression are common COMORBIDITIES in IBS, a psychiatric history is NOT a Rome IV diagnostic criterion. Diagnosis rests purely on pain + stool criteria, not on the presence of psychiatric disease.

BOARD TRAP

"Psychiatric background is a Rome IV criterion" → wrong; it is an associated comorbidity, never part of the diagnostic criteria.

9. Discomfort thrown out (red X)

WHAT YOU SEE

A figure crossed out by 'DISCOMFORT' with 'PAIN ONLY' stamped beside it.

WHAT IT ENCODES

Rome IV DROPPED 'discomfort' as a qualifying symptom — it is PAIN ONLY now. Vague bloating or discomfort without true pain no longer satisfies the criterion. This is a frequently tested Rome III→IV change.

BOARD TRAP

"Abdominal discomfort counts toward IBS" → wrong; Rome IV removed 'discomfort' — pain is required.

10. Painless diarrhea/constipation (red X)

WHAT YOU SEE

Figures on the far right crossed out with red X, distressed and clutching paper.

WHAT IT ENCODES

Painless diarrhea or painless constipation does NOT qualify as IBS because the mandatory pain criterion is absent. Such cases point to functional diarrhea, functional constipation, or an organic cause needing evaluation.

BOARD TRAP

"Painless diarrhea/constipation is IBS" → wrong; without abdominal pain it cannot meet Rome IV.
